

Public Health Emergency Operations Center

"COUSP DRC"

MVE B-17 Incident Management System

Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°077/MVE B_30/07/2026

Affected provinces (5)	HAUT-UELE, ITURI, NORTH KIVU, SUD-KIVU & TSHOPO
Affected Health Zones (49)	• Haut-Uélé (5/13): Boma Mangbetu, Isiro, Pawa, Rungu and Wamba • Ituri (28/36): Aru, Aungba, Bambu, Bunia, Damas, Drodoro, Gethy, Kilo, Komanda, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba, Rwampara, Tchomia, Kambala, Nia-Nia, Fataki, Mandima, Lolwa, Boga, Ariwara, Mahagi and Adja. • North Kivu (11/34): Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha, Vuhovi, Mabalako, Masereka and Musienene. • South Kivu (1/34): Miti-Murhesa • Tshopo (5/23): Kabondo, Lubunga, Makiso-Kisangani, Mangobo and Wanierukula
Reporting date	July 30, 2026
Publication date	July 31, 2026

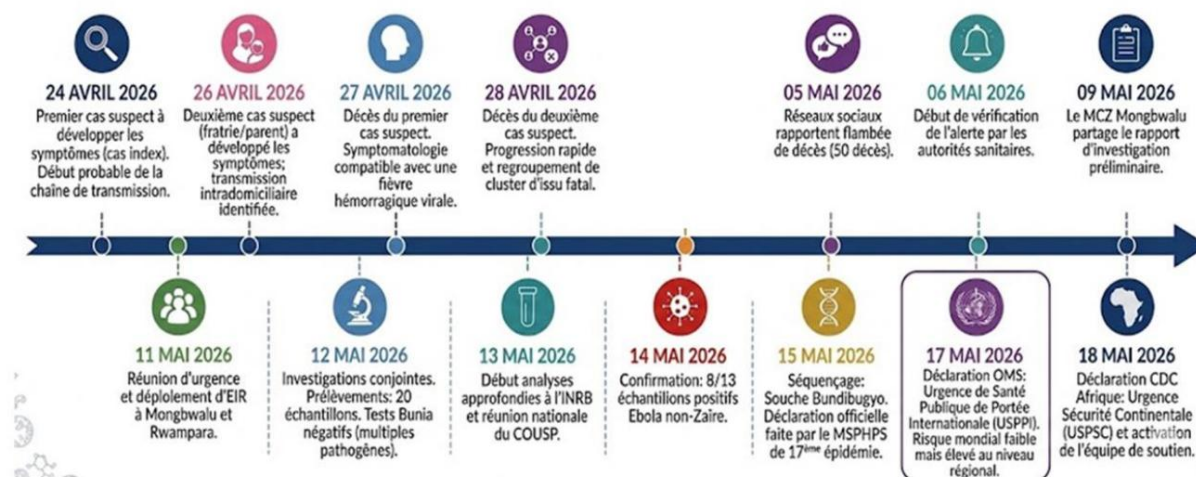
INDICATORS KEYS — CURRENT TO JULY 30, 2026

CONFIRMED CASES — 5 PROVINCES 3,605 +73 new cases (Ituri 52 · N-Kivu 15 · Haut-Uélé 6)	CONFIRMED DEATHS · LEATHITIS 1,587 (44.0%) +24 deaths (Ituri 20 · North Kivu 4) + 7 harmonized (Haut-Uélé)	SUSPECTED CASES OF THE DAY 374 Ituri 222 · N-Kivu 101 · Haut-Uélé 27 · Tshopo 24
PATIENTS IN ISOLATION 748 Ituri 523 · N-Kivu 202 · Haut-Uélé 22 · Tshopo 1	CURED — CUMULATIVE 651 +25 cured (Ituri 25)	NATIONAL CONTACT TRACKING 75.3% 17,828/ 13,420 views · Ituri 75.5% - N-Kivu 74.7% - H-Uélé 80.6% - Tshopo 66.2%

1. Epidemiological Highlights

- **Epidemiological situation:** No new health zones affected in the last 24 hours. The total remains at 49 affected health zones out of 140 across the five provinces. Tshopo province reports a fifth affected zone, Wanie-Rukula, where the 7th provincial case was confirmed on July 29; this classification is not included in the cumulative totals pending harmonization with the provincial health division.
- **Case trends:** In the last 24 hours, 77 new confirmed cases, 24 confirmed deaths and 25 recoveries were recorded.
 - o Ituri: 52 new confirmed cases have been reported, mainly in Bunia (34), Rwampara (15), Aungba (3), Lita (2), Mongwalu (1), Damas (1), Kilo (1), Nizi (1), Mangala (1), Bambu (1), NiaNia (1) and Rimba (1), as well as 20 deaths, 35% of which were community-acquired. (10 new cases were also reported, bringing the daily total to 62)
 - North **Kivu:** 15 new confirmed cases were reported in Katwa (8), Butembo (4), Musienene (2), and Kyondo (1), as well as 4 deaths, all community deaths. The provincial total reaches 364 confirmed cases and 254 deaths, representing a case fatality rate of 69.7%.

Declared on May 15, 2026, the 17th Ebola virus disease (EVD) outbreak caused by the Bundibugyo ebolavirus continues to evolve within a complex humanitarian and security context. Initially confined to the Mongbwalu health zone (Ituri), it The outbreak has spread to five provinces (Ituri, North Kivu, South Kivu, Haut-Uélé, and Tshopo), now affecting 49 health zones. Population movements, insecurity, artisanal mining activities, and cross-border trade with Uganda and South Sudan continue to facilitate transmission. Multisectoral coordination remains mobilized to strengthen surveillance, intensify response efforts, and limit the spread of the epidemic.



2. Descriptive Epidemiological Analysis

TABLE 1 — DISTRIBUTION OF CONFIRMED CASES AND DEATHS BY PROVINCE

Province	Confirmed cases	Death	Lethality	Affected health zones	New cases (24 h)
Haut-Uélé	53	28	52.8%	5/13 (38.5%)	6
Tshopo	7	5	71.4%	4/23 (17.4%)	0
South Kivu	3	1	33.3%	1/34 (2.9%)	0
Total	3,605	1,587	44.0%	49 out of 140	73

Ituri province remains the epicenter of the 17th Ebola virus disease (EVD) outbreak, accounting for 88.1% of confirmed cases (n = 3,176) and 82.6% of deaths (n = 1,311), resulting in a case fatality rate of 41.3%. The health zones of Bunia (880 cases), Rwampara (627), Mongbwalu (541 cases), Nizi (377), Lita (131 cases), and Nyankunde (114) remain the most affected, accounting for approximately 84.1% of reported cases at the provincial level and 74.1% at the national level.

2.2 Distribution by health zone

TABLE 2 — CONFIRMED CASES AND DEATHS BY PROVINCE AND HEALTH ZONE (CUMULATIVE AND CURRENT SITUATION)

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	3,176	1,311	41.3%	52	7	13	20
Adja	11	1	9.1%	0	0	0	0
Aru	5	5	100.0%	0	0	0	0
Ariwara	7	2	28.6%	0	0	0	0
Aungba	9	5	55.6%	3	1	0	1
Bamboo	60	12	20.0%	1	0	0	0
Boga	1	1	100.0%	0	0	0	0
Bunia	880	257	29.2%	32	4	5	9
Damascus	17	8	47.1%	1	0	0	0
Drodro	10	6	60.0%	0	0	1	1
Fataki	27	12	44.4%	0	0	0	0
Gethy	1	0	0.0%	0	0	0	0
Kambala	2	0	0.0%	0	0	0	0
Kilo	29	11	37.9%	1	0	0	0
Komanda	36	25	69.4%	0	0	0	0
Lita	131	75	57.3%	2	0	3	3
Logo	9	3	33.3%	0	0	0	0
Lolwa	7	2	28.6%	0	0	0	0
Mahagi	4	1	25.0%	0	0	0	0
Mambasa	9	6	66.7%	0	0	0	0
Mandima	16	12	75.0%	0	0	0	0
Mangala	101	64	63.4%	1	0	0	0
Mongbwalu	541	258	47.7%	1	0	1	1
Nia-Nia	95	50	52.6%	1	0	1	1
Nizi	377	179	47.5%	1	1	0	1
Nyankunde	114	31	27.2%	0	0	0	0
Rimba	9	4	44.4%	1	0	0	0
Rwampara	627	257	41.0%	7	1	2	3
Tchomia	41	24	58.5%	0	0	0	0
North Kivu	366	242	66.1%	15	4	0	4
Blessed	50	41	82.0%	0	0	0	0
Butembo	82	64	78.0%	4	1	0	1
Goma	1	0	0.0%	0	0	0	0
Kalunguta	5	3	60.0%	0	0	0	0
Katwa	166	100	60.2%	8	2	0	2
Kyondo	13	8	61.5%	1	1	0	1
Mabalako	1	0	0.0%	0	0	0	0
Masereka	4	0	0.0%	0	0	0	0

Musienene	39	22	56.4%	2	0	0	0
Oicha	4	3	75.0%	0	0	0	0
Vuhovi	1	1	100.0%	0	0	0	0
South Kivu	3	1	33.3%	0	0	0	0
Miti-Murhesa	3	1	33.3%	0	0	0	0
Haut-Uélé	53	28	52.8%	6	0	0	0
Boma	12	4	33.3%	4	0	0	0
Mangbetu	16	9	56.2%	0	0	0	0
Isiro	9	7	77.8%	20	0	0	0
Pawa	1	0	0.0%	0	0	0	0
Rungu	15	8	53.3%	0	0	0	0
Tshopo	7	5	71.4%	0	0	0	0
Kabondo	1	1	100.0%	0	0	0	0
Lubunga	1	0	0.0%	0	0	0	0
Makiso-	4	3	75.0%	0	0	0	0
Kisangani	1	1	100.0%	0	0	0	0
Mangobo	1	1	100.0%	0	0	0	0
Total	3,605	1,587	44.0%	73	11	13	24

2.3 Mapping of confirmed cases

The spatial distribution confirms a transmission mainly concentrated in Ituri, particularly in the health zones of Bunia, Rwampara, Mongbwalo, Nizi, Lita and Nyankunde, with transmission continuing in the areas already affected without any new geographical expansion in the last 24 hours.

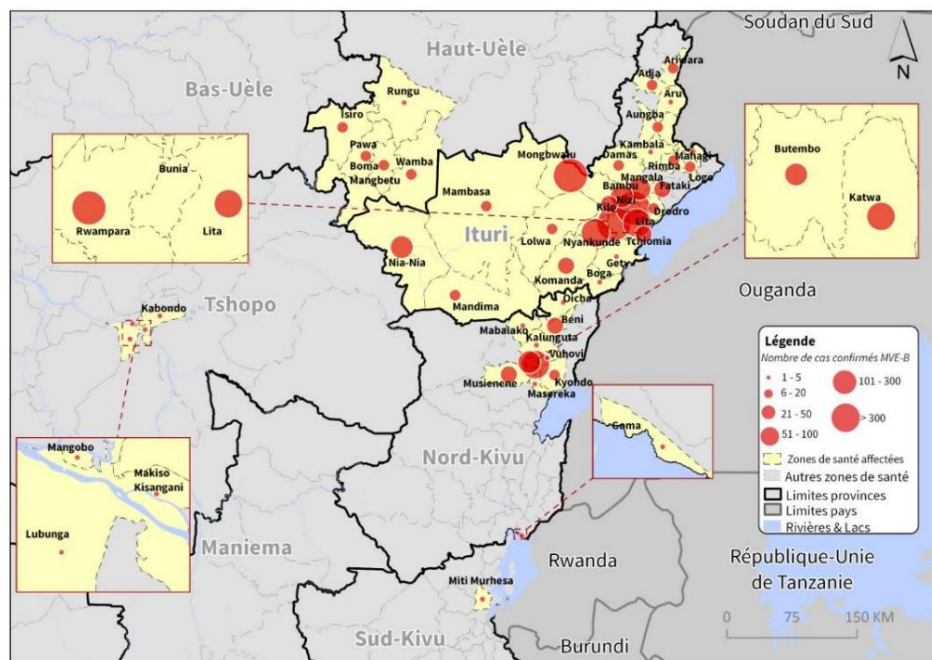


FIGURE 2 — SPATIAL DISTRIBUTION OF HEALTH ZONES THAT HAVE REPORTED CONFIRMED CASES

Transmission in already affected health zones underscores the need to simultaneously maintain intensive interventions at the epicenter and preparedness activities in neighboring provinces.

2.4 Temporal analysis — epidemic curves

The epidemic curve shows sustained transmission during weeks S26 to S31, with fluctuations related to notification delays and the gradual consolidation of data.

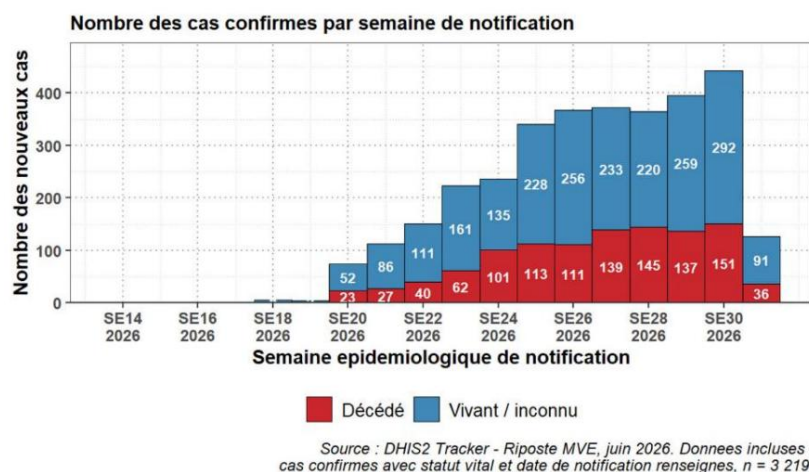


FIGURE 3 — EPIDEMIC CURVE BY WEEK OF NOTIFICATION

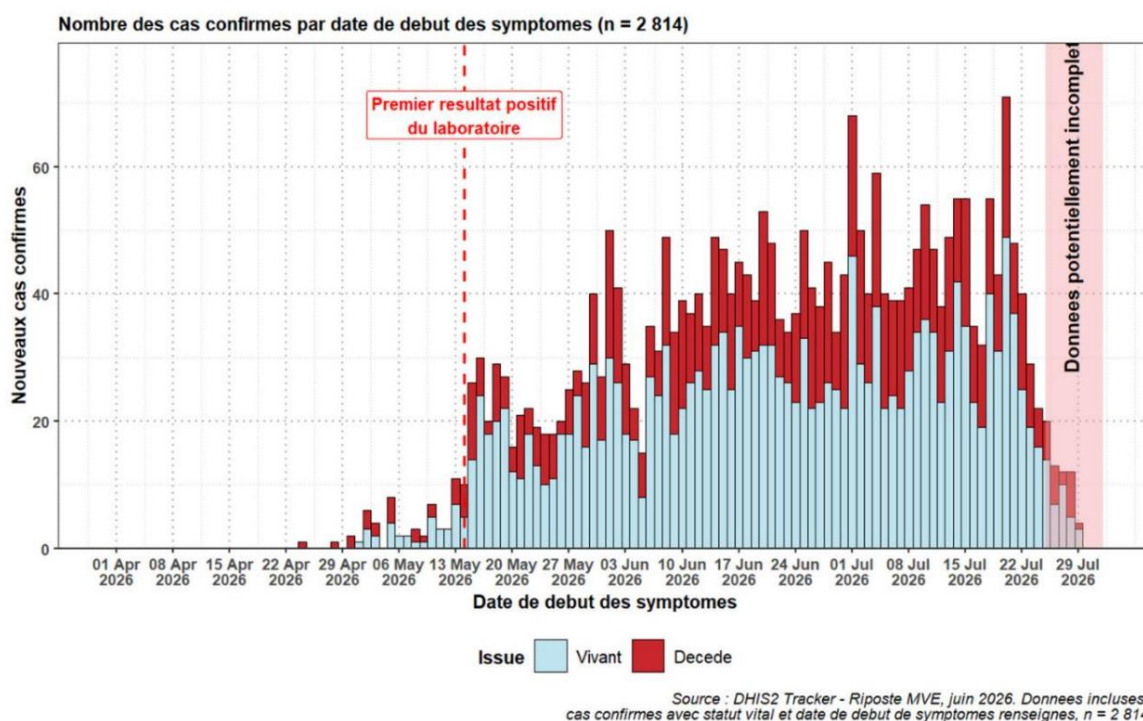


FIGURE 4 EPIDEMIC CURVE BY DATE OF SYMPTOM ONSET (LINEAR LIST DHIS2)

The epidemic remains in a phase of sustained transmission, with a high level of virus circulation. Recent fluctuations should be interpreted with caution due to reporting delays and the gradual consolidation of data.

2.5 Demographic Characteristics

The age pyramid shows a predominance of **working-age adults (18–50 years)**. Minors under 18 and people over 50 are proportionally less affected.

FIGURE 5 — CONFIRMED CASES BY SEX AND AGE GROUP

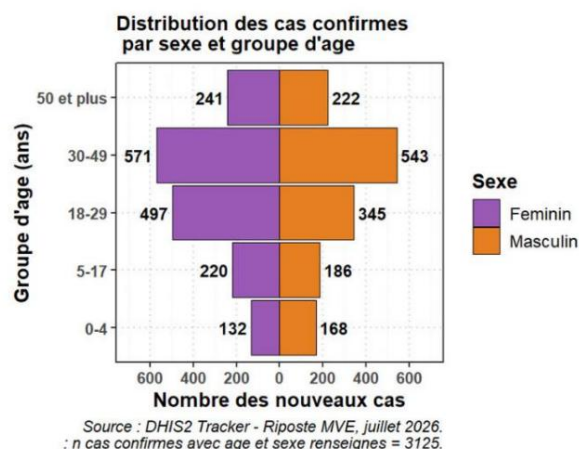
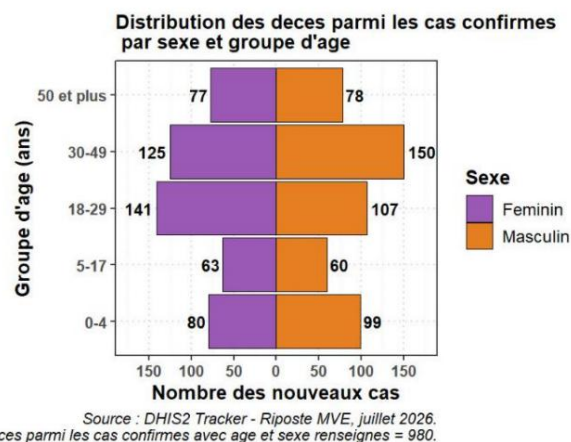


FIGURE 6 — DEATHS BY SEX AND AGE GROUP (AMONG CASES) (CONFIRMED)



The distribution of cases by sex remains balanced. The predominance of cases in working-age adults reflects more frequent community and occupational exposures, while the proportion of pediatric cases testifies to the persistence of intrafamilial transmission.

3 Monitoring, testing and reporting

3.1 Alert Management and Sources

TABLE 3 — MANAGEMENT OF EPIDEMIOLOGICAL ALERTS

Indicator	Ituri	North Kivu	Haut-Uélé	Tshopo	South Kivu	Total
Report (D-1)	10	38	22	0	ND	70
New alerts received	514	875	5	71	ND	1,465
Total alerts for the day	524	913	27	71	ND	1135
Alerts investigated	367	873	27	24	ND	1,291
Investigation rate (24 h)	70.04% 95.62%		100.0%	33.8%	ND	84.10%
Alerts confirmed — live	166	63	27	24	ND	280
Alerts confirmed — deceased (comm.)	56	38	0	0	ND	94
Total suspected cases for today	222	101	27	24	ND	374

In total, 1,535 alerts (including 1,465 new ones) were received, with 1,291 investigated within 24 hours (84.10%) and 374 validated (280 active cases and 94 community deaths). North Kivu accounts for the majority of the volume with 913 alerts investigated at 95.6%, while Ituri saw a decrease to 70.04%, and Tshopo investigated only 24 of its 71 alerts (33.8%). South Kivu did not submit any reports, and its indicators remain unavailable.

3.2 Laboratory: tests and positivity

Ituri : 241 samples documented in the collection network, of which 62 were positive (55 living and 7 deceased), i.e., a positivity rate apparent of 25.7%.

North Kivu: 147 samples received and analyzed (Butembo 91, Beni 56), among which 15 were positive (11 living and 4 dead), representing a positivity rate of 10.2%; 26 analyses remain in progress.

ŷ **Haut-Uélé:** 6 positive results reported, all blood tests (4 from Boma Mangbetu and 2 from Isiro, including one repeat sample); no positive swab results or invalid results. The number of samples received and analyzed is unusable, as the laboratory pillar indicator slide is empty. Only one of the thirteen planned laboratories is operational, at the University Clinics of Uélé in Isiro; the facilities in Wamba and Boma Mangbetu are scheduled to open, and a 10 kW solar kit has been received from the WHO.

ŷ **Tshopo:** 3 samples collected in the Lubunga health zone, results pending; the provincial total stands at 7 positive out of 47 tests. Non-compliant samples are reported in the Lubunga and Opienge health zones, and samples continue to arrive at the laboratory without an investigation form.

3.3 Contact tracking

TABLE 4 — CONTACT TRACKING OF CONFIRMED CASES

Province	Contacts under follow-up	Contacts viewed	tracking rate
Ituri*	11,638	8,781	75.5%
North Kivu*	5,667	4,227	74.6%
Tshopo	65	43	66.2%
Haut-Uélé	458	369	80.6%
South Kivu (end of monitoring)	0	0	N / A
Total	17,863	13,455	75.3%

* In Ituri and North Kivu, the monitoring rate was calculated only for health zones that reported. The health zones of Kambala and Mahagi (Ituri) and Rungu (Haut-Uélé) did not have usable data at the time of report consolidation.

Contact tracing remains insufficient at the national level (75.3%), below the operational threshold of 95%, and has declined by 3.4 percentage points compared to the previous day. Ituri only managed to trace 8,781 contacts out of 11,638 (75.5%), with three areas at 0% (Mahagi, Kambala, and Boga) and very low performance in Nyankunde (12.7%) and Bunia (22.0%), where follow-up data was not reported. North Kivu recorded 4,262 contacts seen out of 5,702 (74.7%). Haut-Uélé reported 369 contacts seen out of 458 (80.6%), and Tshopo improved to 66.2% (43 contacts seen out of 65), with the Kabondo area remaining at 0%.

3.4 Entry points and control points (PoE/PoC)

3.4.1 — Crossing point alerts (24 h)

Ituri — 3 alerts notified

All three alerts were notified to the Mud'zibala PoC and concern travelers bound for Bunia: a man

A 29-year-old man with fever (37.7°C) presented with diarrhea, vomiting, myalgia, and abdominal pain. A 25-year-old woman and a 25-year-old man were also identified. All three were isolated, assessed, and referred to the COVID-19 Treatment Center (CTE) at Bunia General Referral Hospital (HGR). No bodies were recovered. Two PoCs, Abasina (Nia-Nia Health Zone) and Irumu Centre (Nyankunde Health Zone), did not transmit their data, with completeness set at 96.7% (58/60).

North Kivu — 0 alerts notified

No alerts were notified to the sixteen PoE/PoCs in the province, all of which reported (completeness and promptness 100%).

No lifeless bodies were intercepted. Of the 60,233 travelers checked, 1,633 refused screening and handwashing, or 2.7%, with the highest proportion recorded at the Mususa PoC in the Musienene health zone (25%).

Tshopo — 0 alerts notified

No alerts were reported. The data available at the time of consolidation covers July 29: 2,348 travelers were screened at the Bangboka Point of Entry (PoE) and the ONATRA and PK 23 RN4 Points of Control (PoCs), of whom 97.7% were screened and 97.2% washed their hands. The PK 23 RN3 PoC again did not report its data.

Haut-Uele — 0

No alerts were notified to the activated PoE/PoC points in the province. Eight of the ten activated points reported (80% completeness), for 16,536 travelers checked, fully screened, washed and sensitized; the Apodo (Gombari) PoC alone accounts for 3,177 passages, or 19% of the provincial total.

3.4.2 Monitoring indicators for PoE/PoC

TABLE 5 — MONITORING OF INDICATORS AT POINTS OF ENTRY AND CHECKPOINTS (POE/POC)

Indicators	Ituri	North Kivu	South Kivu	Tshopo Haut-Uélé	Overall
Proportion of PoC enabled and PoE enhanced	100% (60/60)	100% (16/16)	100% (9/9) 21.1% (4/19)	30%*	85.6%*
Number of people who have switched to PoE/PoC	115,981	60,233	ND	2,348 16,536	195,098
% of travelers screened	96.7%	97.3%	96.33%	97.7% 100% 97.2%*	
% of travelers who washed their hands	96.5% 97.3%		96.33%	97.2% 100% 97.1%*	
% of travelers made aware	99.1% 97.3%		100%	100% 100% 98.7%*	
Number of alerts notified	3	0	0	0 0	3
Number of validated live alerts	3	0	0	0 0	3
Number of bodies recovered today	0	0	0	0 0	0
Number of problematic contacts intercepted today	0	0	0	0 0	0
Percentage of validated live alerts referred to the CT/CTE	100% (3/3)	0	0	0 0 100% (3/3)	
% of lifeless bodies swabbed	ND	0	0	0 0	ND
Number of positive results of referred suspected cases	0	0	0	0 0	0

4. Support

TABLE 6 — OCCUPANCY OF HEALTHCARE FACILITIES (CTE/CT/CI)

Indicator	Ituri	North Kivu Haut-Uele	Tshopo Together	
Number of beds	820	141	68	20 1049
Patients in bed (Day -1)	553	220	25	1 799
Total admissions (24 hours)	58	53	7	0 118
Total admissions	771	321	47	1 1,140
Deceased	17	2	0	0 19
No case	44	67	0	0 111
Healed	25	0	0	0 25
Escapes	2	0	0	0 2
Transferred to the HGR	0	1	0	0 1
Total outflows (24 h)	88	70	0	0 158
Patients in isolation (end of day),	523	202	22	1 748
including confirmed cases	301	25	10	1 337
including suspects	222	177	12	0 411
Occupancy rate	63.8%	143.3%	32.4%	5.0% 71.3%

In total, 748 patients were in isolation, including 337 confirmed cases and 411 suspected cases, for an overall occupancy rate of 71.3% (748 patients out of 1,049 beds), with 118 admissions and 158 discharges (25 recovered, 19 deaths, 111 non-cases, 2 escaped, and 1 transferred). Ituri accounts for the majority of the burden with 523 patients in isolation (301 confirmed, 222 suspected) out of 820 beds, representing an occupancy rate of 63.8%. In North Kivu, 202 patients are in isolation (25 confirmed, 177 suspected) for an occupancy rate of 143.3% (141 beds, capacity not reported for four cycles). Haut-Uélé has 22 patients in isolation out of 68 beds distributed across five Ebola Treatment Centers (32.4%), with the Wamba Ebola Treatment Center exceeding its capacity for confirmed cases (125%). Tshopo has 1 patient in isolation out of 20 beds (5.0%). Despite an overall occupancy rate of 63.8% in Ituri, six facilities remain above or at the limit of their capacity.

for confirmed cases: CTE Nizi (322%), CTE Lita (140%), CTE ISTM (107%), CTE Bunia (105%), CTE CME (100%) and CTE Fataki (100%); three CTs are saturated for suspected cases: Logo (125%), Kigonze (113%) and HGR Kilo (100%).

5 Mental Health and Psychosocial Support (MHPSS)

TABLE 7 — KEY SMSPS INDICATORS

SMSPS Key Indicators	Ituri	North Kivu
New confirmed cases that benefited from SMSPS interventions	8 (15.4%)	ND
Confirmed cases are being monitored.	136 (45.2%)	ND
New suspected cases	20 (12.0%)	ND
Suspected cases are being monitored	90 (40.5%)	ND
Laboratory results announced (including positive ones)	36 (8)	ND
Children separated — nursery / community	0/0	ND
CTE support staff	222	ND
Frontline Personnel (PPL)	27	ND
Members of households and communities	90	ND
EDS supported (SMSPS)	18	ND

6 Frontline Personnel (PPL) assigned

TABLE 8 — FRONTLINE WORKERS (FWs) INFECTED WITH EVD IN ITURI

Health Zone	Confirmed cases	Cured.	Hospitalized.	Death
Bunia	35	19	1	14
Rwampara	33	25	1	7
Mongbwalu	17	6	1	9
Nyankunde	14	11	1	2
Nia-Nia	20	1	13	4
Lita	4	1	2	1
Nizi	3	0	0	3
Mangala	4	2	1	1
Bamboo	2	2	0	0
Kilo	2	1	1	0
Logo	3	0	3	0
Total — case fatality rate 29.9%	137	68	24	41

PPL = healthcare personnel and frontline providers.

In North Kivu, 11 cases of PPL are confirmed, including 3 deaths; Haut-Uélé reports 3 (5.7% of its confirmed cases), one of whom is hospitalized at the Isiro CTE. Table 8 remains unchanged: the Ituri provincial report of July 30 does not include a breakdown of PPL cases by health zone and reports no new positive PPL cases that day.

7 Pillar-Based Response Activities

COORDINATION Holding of the coordination meeting for the response and monitoring of activities.	CONTINUITY OF ESSENTIAL SERVICES (CSSE) ÿ Ituri: Continuation of the assessment of free healthcare, phase 1: the free healthcare unit is established (PTF, INSP, DPS, CSSE pillar, EUP Fass, CDR), hotspot zones and structures are prioritized, the free healthcare package is defined, and the partner mapping is complete. 163 health establishments have been pre-selected in 137 health areas across 36 zones, including 92 state-run, 48 faith-based, and 23 private; 43 of them already receive support from a partner. The costing establishes a monthly requirement of USD 2,209,095 to USD 3,976,371 (77 ESS PCAs for USD 1,842,329 and 86 ESS PMAs for USD 366,766). The official directive prohibiting any payments at the targeted healthcare facilities and the contracting with stakeholders remain unimplemented. Phase 2, "Health Security," includes triage at the entrance, a separate circuit for suspected cases, and rapid SOP/MEGS/PCI training.
LOGISTICS ÿ Ituri: 27 missions were carried out, 26 of which were successful, for 27 patients evacuated. Two mattresses were donated to the Rwankole Health Center with WHO support. The laboratory pillar requests four vehicles for supplies, sample transport, and supervision. A used 20 KVA generator and a computer kit (computer, multitasking printer, overhead projector) were received from UNICEF for the Surveillance, IPC, and CREC pillars. IPC and PECH kits were delivered to the health zones of Aungba, Kambala, Lolwa, Mambasa, Angumu, Fataki, Damas, and Rimba; IPC kits were delivered to the SGI depot and the Nia-Nia CTE; and PECH equipment was delivered to the Bunia CME CTE. Africa CDC provided one standard Starlink and five mini Starlink units. The fleet consists of 38 vehicles and 10 ambulances distributed among the pillars and nine health zones. 320 liters of fuel were provided to the Safety and Security pillar. Of the 26 ambulances identified, 23 are available and 3 are out of service (1 in Bambu, 1 with the FARDC, and 1 in the Rwampara Health Zone); 33 trips were made, 29 of which were successful, evacuating 25 patients, with 4 refusals in Bunia. Work is underway at the Rwankole Treatment Center (tiling, paving, and electrical installation), and expansion tents are being erected at the Nia-Nia Treatment Center. ÿ North Kivu: Delivery of IPC supplies to the Musienene Health Zone; receipt of medicines (UG-PDSS donation) and dignity kits and buckets (UNFPA donation); deployment of pillar teams to health zones. Logistics indicators remain poor: 50 motorcycles	PCI / WASH ÿ Ituri: Training began for 50 healthcare providers and hygiene workers in the Nizi health zone with support from FHI 360; distribution of Category 3 IPC-WASH kits to three health facilities (CBCA Health Center, CNCA Health Center, and Bigo Health Center) with support from PPSSP/UNICEF, and 10 handwashing stations at the Governorate 2 market (FAEVU support). 25 rings opened around cases and 28 sites identified, all decontaminated within 48 hours, representing a 100% implementation rate; decontamination of 27 of the 69 households with cases within 48 hours (39.1%), mainly in Nizi (14), Bunia (5), Rwampara (3), Mongbwalu (2), and Mandima (1); 39 community health workers monitored and supported (Bunia 25, Nizi 7, Rwampara 7) out of a target of 394; 180 providers trained (Bunia 140, Bambu 27, Drodoro 13). The completeness of IPC reporting remains low, at 10 out of 29 health zones (34%). ÿ North Kivu: Continued training of 20 IPC/WASH focal points and 20 supervisors in the Kalunguta Health Zone with WHO support; decontamination of one household of 14 confirmed cases in Musienene and of three health facilities (Katwa 1, Musienene 2); scorecard assessment of one health facility in Beni, average score 51.6%; monitoring and support of 67 health facilities (Beni 38, Oicha 24, Katwa 3, Kyondo 2) and briefing of 240 service providers during formative supervision sessions. Nine staff exposure risk assessments were conducted, eight of which were high-risk. Only 4 out of 11 areas were completed. Tshopo: PCI assessment of the Konga Konga displaced persons site, score 19.4%, decontamination of the site and the

Functional out of 100 expected, 6 out of 7 vehicles, 5 out of 30 ambulances, no hearses out of 20, 2 out of 11 CTEs built according to standards, and no CTs out of 36; no area is free from shortages of IPC inputs, and only 4 out of 11 areas are supplied with holistic care inputs. Tshopo: water supply to the PoC PK 23, packaging and delivery of an IPC kit to the Wanie-Rukula Health Zone, visit to the construction site of the large-capacity CTE with COUSP, IMA, and MSF; the fleet remains limited to 4 liaison vehicles, 4 motorcycles, and 4 ambulances when the requirement is 10 vehicles and 27 motorcycles, and Village Reach has delivered only 500 of the 1,000 liters of diesel promised.	FONAREV dispensary, handover of an PCI kit and mattresses to CS Madula and briefing of service providers on water triage and chlorination.
COMMUNICATION (RCCE / CREC) ÿ Ituri: 2,717 people reached during 5 home visits, 2,550 during 12 educational talks, 5,325 during 13 mass awareness campaigns, 4,182 in churches, mosques, and markets, and 1,282 during 10 community dialogues; 12 people engaged following advocacy efforts. Mass awareness campaign of approximately 300 people at the Hoho market (Rwampara Health Zone) to demystify body bags and chlorinated water through practical demonstrations, with support from WHO, REMED, Africa CDC, UNICEF, and FHI 360. 104 community alerts reported (100 living, 4 deaths); 14 instances of resistance recorded, none overcome, all in the Bunia Health Zone; 35 feedback points identified, 5 of which were clarified (25%). Capacity building: 35 young leaders in Mambasa (IMA), 26 IT, ITA and ECZ members in Mongbwalu (Africa CDC), 30 RECO in Nizi (MEDAIR, FHI 360) and 42 Caritas agents in Bunia (CRS funding). Two radio programs produced, six spots broadcast, 36 community radio stations involved. ÿ North Kivu: The mayor of Bungulu commune is involved in raising awareness among his community about prevention measures and the safety of intervention teams. A briefing was held for 17 managers of private health facilities in Rwenzori commune (Beni Health Zone) on CREC (Community-Based Respiratory Chemotherapy), community-based surveillance, and standard precautions. Capacity building was provided to 35 writing club facilitators, members of Village Savings and Loan Associations (VSLAs), and psychosocial workers in Vuhovi. A prevention radio broadcast was broadcast in Njiapanda (Biena Health Zone) addressing 76 listener concerns. A community dialogue was conducted in Itebero with leaders and managers of mining sites along the forest and park corridors.	HOLISTIC CARE (HCC) ÿ Ituri: 25 new recoveries; 523 patients in isolation (301 confirmed, 222 suspected), 58 new admissions including 3 advanced or hemorrhagic cases, and 88 discharges including 17 deaths, 44 non-cases, and 2 who escaped; 993 hot meals served (139 confirmed, 129 suspected, 690 contacts, and 35 people with primary AIDS), 20 specific diets, and 8 infants under 6 months receiving infant formula. The cumulative number of recoveries has reached 598. A pillar meeting with partners was held, patients were transferred from ESS, CT, and the community to CTEs, results were announced, and the expansion and construction work on the CTEs continued; training of providers in GIH-mhGAP and monitoring of PECH data entry in DHIS2 at the HGR CTE continued. from Bunia. ÿ North Kivu: Cumulative total of 142 confirmed cases admitted to Ebola Treatment Centers (ETCs) with 48 deaths, representing an ETC mortality rate of 34%, and 13 lost to follow-up. 5 confirmed and 48 suspected cases were admitted today, with 2 deaths, 1 transfer, and 67 non-cases discharged, leaving 202 patients in isolation at the end of the day. Hot meals were served to 74 confirmed cases, 328 suspected cases, 100 frontline healthcare providers, and 294 caregivers. 152 households received general food distribution support. One separated infant is receiving breast milk substitute care in Kalunguta. Training was conducted for isolation staff at Beni General Referral Hospital and the Bundji and Boikene Health Centers with support from IMC. An assessment mission was carried out in Musienene, and joint APATEMACO/WFP supervision was conducted in Butembo and Katana. ÿ Haut-Uélé and Tshopo: No deaths or recoveries in Haut-Uélé, where 2 confirmed cases were admitted (Boma Mangbetu 1, Wamba 1) and 5 suspected cases (Wamba 3, Pawa 2), for a total of 22 patients hospitalized at the end of the day; the Wamba Ebola Treatment Center is exceeding its capacity for confirmed cases (125%) and the Ebola Treatment Center of

national of Kahuzi-Biega. 20,825 households visited and 33,785 people reached door-to-door, 4,402 in public places and 1,271 during dialogues and talks; 1,199 people engaged and 210 alerts of suspected cases reported by community actors; all 34 targeted radio stations broadcast. ŷ Tshopo: Pillar meeting with situation analysis and deployment of teams by area (12 participants); advocacy with authorities and influencers in Kisangani for contact tracing of the 7th case (15 participants); focus group with managers of the Konga Konga IDP camp (27 participants); community dialogue and awareness-raising at the IDP camp, reaching 867 people. Support for training rapid response teams, follow-up to 7 community alerts in Mangobo, and continued dissemination of public service announcements. Haut-Uélé: continued mass awareness campaigns in churches, reaching 3,816 people through home visits in 1,272 households, 1,426 through mass awareness campaigns, and 63 through discussions; 3 of the 14 targeted radio stations broadcast; commencement of translation of IEC tools into Mayogo and Mangbetu.	Boma Mangbetu is understaffed. Visits were made to the University Clinics, Libiki Hospital, and the Tuluba Health Center, where a PPL (Public Health Program) representative was accompanied. Tshopo: no admissions or discharges, only one confirmed case hospitalized at the Kisangani Hospital Center, stable on the second day; experimental treatment continues for 3 high-risk contacts among frontline providers at the Madula Health Area (Wanie-Rukula Health Zone); the cumulative number of hospitalized confirmed cases is 4, including 2 deaths, 1 recovery, and 1 in intensive treatment course.
RESEARCH, INNOVATION & VACCINATION ŷ Ituri: Launch of Africa CDC's research activities by the Incident Manager, Professor Steve Ahuka. Pillar meeting and training of enumerators for the rapid qualitative survey with support from Africa CDC. Final session analyzing the challenges of the nine key pillars of the response and validation of the recommendations formulated, with support from WHO. Outlook: launch of the community survey after the enumerator briefing, organization of two conferences within the university community of Bunia, and continued receipt of partner protocols for analysis and certification. No vaccination activities were reported by the provinces today.	Dignified and secure burials (DSM) ŷ Ituri: 3,061 alerts were recorded for 2,496 EDS (Emergency Health Diagnosis) assessments carried out between May 20 and July 30. There were 72 new alerts and 24 postponements, with 54 EDS assessments completed (56%), 12 failures, and 31 postponed to the following day. 42 swabs were conducted, including 30 in communities and 12 in healthcare facilities. The alerts were distributed as follows: 47 community and household alerts, 14 in social and solidarity economy (SSE) settings, and 8 in treatment centers. Training began for two EDS teams in the Tchomia health zone, and training for the two teams in the Mangala health zone was completed. 18 EDS staff received psychosocial support. ŷ North Kivu: 33 death alerts received, 33 bodies swabbed and secured, and 31 DHS (Death Health Survey) carried out, without any failures; 16 swabbed and secured bodies remain pending. Deaths occurring in the community constitute the majority of alerts, primarily in Beni (14) and Katwa (12). 34 community deaths benefited from a DHS and 43 were swabbed.
PREVENTION OF SEXUAL EXPLOITATION AND ABUSE (PSEA / PRSEAH) ŷ Ituri: Briefing and awareness-raising began with focal points in the Fataki and Mandima health zones, and cascade briefings continued in the Ariwara health zone with WHO support. 62 people were briefed (56 men, 6 women), bringing the total to 2,659 people who have signed the commitment agreement in seven health zones, and 477 people were sensitized (223 men and boys, 254 women and girls), bringing the total to 15,806. Twelve Community-Based Community Management (CBCM) units were revitalized in Mongbwalu.	

Security and Community Acceptance: Several incidents and situations of resistance marked this period. In North Kivu, on July 29 at 5:30 p.m., the team responsible for dignified and safe burials, accompanied by security services, was attacked in Kasitu, 6 km from Beni on the road to Mangina, during the burial of a confirmed body. The local population handled the body, and warning shots were necessary to disperse the troublemakers, without any material or human damage. At the Mukulya PoE (Police Emergency Response Unit), a police officer suspected of harassment was arrested, and an investigation is underway. A couple being treated at the PNC (Congolese National Police) hospital categorically refused transfer to the HGR (General Referral Hospital) and requested a home sample collection. In Haut-Uélé, a clinically recovered patient from Boma Mangbetu is making threats against healthcare staff: joint CREC (Regional Center for Epidemic Response) actions and care management are underway with the area's chief medical officer within 48 hours, and security for the laboratory and Ebola Treatment Centers (ETCs) by the National Police has only been secured for the laboratory. In Tshopo, no operational follow-up has been documented regarding the forced removal of the body of a confirmed case from the Makiso-Kisangani General Referral Hospital morgue; the community at PK 17 continues to question the movement of the deceased, and contact tracing in Kabondo remains at 0%.

8 Main challenges, impacts and actions required

Challenge No.	Impact	Actions required
1	Reluctance to participate in post-mortem sampling (EDS/swabs), community resistance, and insecurity targeting EDS teams (bridge attack) Muchanga, paralysis of activities in Nyankunde)	Confirmation delays, underestimation of cases, disruption of key activities Intensify CREC, involve religious and community leaders, secure the teams, increase the number of EDS teams
2	Non-payment of service providers (strike in the health zones of Bunia and Rwampara; resumption announced after the intervention of the military governor of Ituri)	Interruption of response activities, including dignified and safe burials Regularize payments to service providers; secure dedicated funding
3	Insufficient capacity of CTE/CT: occupancy of 143.3% in North Kivu (bed capacities: 141, not reported for four cycles) and six structures beyond their capacity for confirmed cases in Ituri, including the Nizi CTE at 322% and the Lita CTE at 140%;	Admission delays, increased risk of nosocomial transmission Accelerate the construction and expansion of CTEs, deploy temporary structures, and systematically report bed capacity.
4	Low and declining performance of contact tracing: Ituri 75.5%, North-Kivu 74.7%, Tshopo 66.2% and Haut-Uélé 80.6%, with three zones at 0% in Ituri (Mahagi, Kambala, Boga) and Kabondo at 0% in Tshopo	Non-transmission chains interrupted Train, deploy and motivate community liaison officers (RECOs); strengthen supervision; investigate the observed drop in Ituri
5	Ambulance shortages and pre-positioning at points of control	Delays in transfer and isolation Preposition ambulances (including those at PoE/PoC), deploy CTs per ZS
6	Insufficient supply of MEG and medical inputs	Weakening of overall care Advocacy and urgent supply of essential medicines
7	Insufficient PCI inputs (PPE, chlorine) and limited functional triage units (181/1170, i.e. 15%)	Risk of nosocomial infections (137 cases of PPL, including 41 deaths in Ituri; 11 PPL, including 3 deaths in North Kivu and 3 PPL confirmed in Haut-Uélé) Urgent supply, strengthening of IPC training

Challenge No.		Impact	Actions required
8	Geographic extension (Haut-Uélé, Tshopo) and high mobility	Risk of urban, riverine and cross-border spread	Activate the 23 priority PoE/PoC points in Haut-Uélé; strengthen cross-border coordination
9	Installation of local laboratories (Rwampara, Fataki, Bambu-mine, Boga and Mambasa)	Long confirmation delays, transmission not detected	Accelerate the deployment of decentralized diagnostic capabilities
10	Financial shortcomings of the resources	Limitation of the implementation of pillars	Urgent mobilization of resources, advocacy with partners
11	Insecurity and limited access (CODECO, ADF)	Restriction of operations	Strengthening security coordination and humanitarian access
12	Continuity of essential services (CEHS) compromised	Increase in indirect (non-Ebola) mortality	Effective implementation of free healthcare, strengthening of CEHS

9 Photos of field activities



PoC Support and Treatment at WANIE RUKULA AT PK23

FOR ANY INFORMATION

**ADDITIONAL INFORMATION, PLEASE
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